TCF induction followed by radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction Day 1 · docetaxel / cisplatin / fluorouracil

Active medicines: Docetaxel + Cisplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Ondansetron	16 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Olanzapine	5 mg PO OD	Day 1 Source-linked draft; reconcile to one authorised prescription before use.
Dexamethasone for docetaxel premedication	8 mg PO BD	For 3 days starting the day before docetaxel; at least 3 doses before treatment unless contraindicated Source-linked draft; reconcile to one authorised prescription before use.
Aprepitant	80 mg PO OD	Days 2-3 Source-linked draft; reconcile to one authorised prescription before use.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Source-linked draft; reconcile to one authorised prescription before use.
Olanzapine	5 mg PO OD	Days 2-4 Source-linked draft; reconcile to one authorised prescription before use.
Reconciled corticosteroid schedule	Prescriber/pharmacist to complete As prescribed As prescribed	Include docetaxel Day −1/Day 1/Day 2 coverage and delayed antiemetic needs; no duplicate dexamethasone Source-linked draft; reconcile to one authorised prescription before use.

Additional phase-specific support

- Docetaxel 75 mg/m² and cisplatin 75 mg/m² on Day 1, with fluorouracil 750 mg/m²/day on Days 1–5. Repeat every 21 days for 4 cycles unless progression or unacceptable toxicity develops.
- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) may be used. After complicated neutropenia, the source recommends prophylactic coverage in subsequent cycles (example Days 6–15). Select the product and schedule individually; this is not an all-cycle mandatory instruction.
- Docetaxel premedication is dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses should precede treatment unless contraindicated. If oral premedication was missed, the source permits dexamethasone 20 mg intravenous (IV) at consultant discretion. This is not automatic permission to proceed. Reconcile the total steroid with antiemetic dexamethasone; do not add both schedules blindly.
- Prescribe cisplatin pre- and post-hydration and electrolytes using current renal/cardiac status and local policy. The national source suggests saline/magnesium with optional potassium, but a reference list is not an individual fluid prescription. Monitor intake, urine output, renal function and electrolytes.
- Document dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) phenotype and/or genotype before first fluorouracil exposure unless previously tested. Complete deficiency precludes fluorouracil; partial deficiency needs an individual starting-dose plan. Renal impairment can elevate uracil and misclassify phenotype; consider genotype. A normal result does not exclude all severe toxicity.
- Docetaxel premedication is dexamethasone 8 mg orally twice daily for 3 days starting the day before docetaxel; at least 3 doses should precede treatment unless contraindicated. If oral premedication was missed, the source permits dexamethasone 20 mg IV at consultant discretion. This is not automatic permission to proceed. Reconcile the total steroid with antiemetic dexamethasone; do not add both schedules blindly.
- Arrange dental/oral assessment, mouth care, dietetic and swallowing support according to need. Review weight, oral intake, pain and hydration throughout chemotherapy and radiotherapy; no automatic feeding-tube decision is generated.

- Preserve the CUH high/moderate and docetaxel proforma adjuncts where applicable: omeprazole, local nystatin/ chlorhexidine mouth care, breakthrough antiemetics and loperamide. These are historical local reference items requiring current-policy and patient-specific review, not universal prophylaxis or an additional duplicate prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00324 v6b · pNCCP 00324: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00324 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required